

Leaflet	Graphic Design beginning - 21.01.2025 Aariya Regulatory Services Pvt. Ltd. - 21.01.2025 (R-1)					
Product name	TachoSil LFT MYS			Reason of change:	Manufacturing site name change and Marketing Authorization Transfer	
Item number identification	6092020 / XXXX (X) / Code XXXX					
Printed colours	Black 100%			prev. version:	Not Applicable	
Not printed colours		dye cut	Perforation: ()	Font sizes	product name: 22 pt text: 9 pt other: 6 pt	
Dimensions	210 x 297 mm					

TACHOSIL MEDICATED SPONGE

Human fibrinogen/Human thrombin



1. NAME OF THE MEDICINAL PRODUCT

Tachosil medicated sponge

2. QUALITATIVE AND QUANTITATIVE COMPOSITION

Tachosil contains per cm²:

Human Fibrinogen 5.5 mg

Human Thrombin 2.0 IU

For a full list of excipients, see section 6.1.

3. PHARMACEUTICAL FORM

Medicated sponge

Tachosil is an off-white sponge. The active side of the sponge, which is coated with fibrinogen and thrombin, is marked by a yellow colour.

4. CLINICAL PARTICULARS

4.1 Therapeutic indications

Tachosil is indicated in adults for supportive treatment in surgery for improvement of haemostasis, to promote tissue sealing, and for suture support in vascular surgery where standard techniques are insufficient (see section 5.1).

4.2 Posology and Method of Administration

Posology

The use of Tachosil is restricted to experienced surgeons.

The number of Tachosil sponges to be applied should always be oriented towards the underlying clinical need for the patient. The number of Tachosil sponges to be applied is governed by the size of the wound area.

Application of Tachosil must be individualised by the treating surgeon. In clinical trials, the individual dosages have typically ranged from 1-3 sponges (9.5 cm x 4.8 cm); application of up to 10 sponges has been reported. For smaller wounds, e.g. in minimal invasive surgery the smaller size sponges (4.8 cm x 4.8 cm or 3.0 cm x 2.5 cm) are recommended.

Method and route of administration

For local use only. Do not use intravascularly.

It is strongly recommended that every time Tachosil is administered to a patient, the name and batch number of the product are recorded in order to maintain a link between the patient and the batch of the product.

See section 6.6 for more detailed instructions.

Paediatric patients

Tachosil is not recommended for use in children below age 18 years due to insufficient data on safety and efficacy.

4.3 Contraindications

Tachosil must not be applied intravascularly.

Hypersensitivity to the active substances or to any of the excipients.

4.4 Special warnings and precautions for use

For local use only.

Do not use intravascularly. Life threatening thromboembolic complications may occur if the preparation is unintentionally applied intravascularly.

Specific data have not been obtained on the use of this product in neurosurgery or in gastrointestinal anastomoses surgery.

Tachosil should not be used for the treatment of severe or brisk arterial bleeding because Tachosil has not been evaluated in this treatment.

Tachosil should not be used in procedures involving the renal pelvis or ureter because it may be a focus for calculus formation.

Tachosil should not be used in the closure of skin incisions since it may interfere with the healing of skin edges or cause wound dehiscence.

As with any protein product, allergic type hypersensitivity reactions are possible. Signs of hypersensitivity reactions include hives, generalised urticaria, tightness of the chest, wheezing, hypotension and anaphylaxis. If these symptoms occur, the administration has to be discontinued immediately.

To prevent the development of tissue adhesions at undesired sites, ensure tissue areas outside the desired application area are adequately cleansed before administration of human fibrinogen/human thrombin sponge (patch) (see *Instructions for Use/Handling*). Events of adhesions to gastrointestinal tissues leading to gastrointestinal obstruction have been reported with use in abdominal surgery carried out in proximity to the bowel.

In case of shock, the current medical standards for shock treatment should be observed.

When medicinal products prepared from human blood or plasma are administered, the possibility of transmitting infective agents cannot be totally excluded. Standard measures to prevent infections resulting from the use of medicinal products prepared from human blood or plasma include selection of donors, screening of individual donations and plasma pools for specific markers of infection and the inclusion of effective manufacturing steps for the inactivation/removal of viruses. Despite this, when medicinal products prepared from human blood or plasma are administered, the possibility of transmitting infective agents cannot be totally excluded. This also applies to unknown or emerging viruses and other pathogens.

The measures taken are considered effective for enveloped viruses such as HIV, HBV and HCV and for the non-enveloped virus HAV. The measures taken may be of limited value against non-enveloped viruses such as parvovirus B19. Parvovirus B19 infection may be serious for pregnant women (fetal infection) and for individuals with immunodeficiency or increased erythropoiesis (e.g. haemolytic anaemia).

It is strongly recommended that every time Tachosil is administered to a patient, the name and batch number of the product are recorded in order to maintain a link between the patient and the batch of the product.

Do not leave Tachosil in an infected or contaminated space because it may potentiate an existing infection.

4.5 Interactions with other medicinal products and other forms of interactions

No formal interaction studies have been performed.

Similar to comparable products or thrombin solutions, the sealant may be denatured after exposure to solutions containing alcohol, iodine or heavy metals (e.g. antiseptic solutions). Such substances should be removed to the greatest possible extent before applying the sealant.

4.6 Pregnancy and lactation

The safety of Tachosil for use in human pregnancy or breastfeeding has not been established in controlled clinical trials. Experimental animal studies are insufficient to assess the safety with respect to reproduction, development of the embryo or foetus, the course of gestation and peri- and postnatal development.

Therefore, Tachosil should be administered to pregnant and breastfeeding women only if clearly needed.

4.7 Effects on ability to drive and use machines

Not relevant.

4.8 Undesirable effects

Clinical Trials

The safety data from clinical trials of human fibrinogen/human thrombin sponge (patch) generally reflect the type of post-operative complications related to the surgical settings in which the trials were conducted and the underlying disease of the patients.

Hypersensitivity or allergic reactions (which may include angioedema, burning and stinging at the application site, bronchospasm, chills, flushing, generalised urticaria, headache, hives, hypotension, lethargy, nausea, restlessness, tachycardia, tightness of the chest, tingling, vomiting, wheezing) may occur in rare cases in patients treated with fibrin sealants/haemostatics. In isolated cases, these reactions may progress to severe anaphylaxis. Such reactions may especially be seen, if the preparation is applied repeatedly, or administered to patients known to be hypersensitive to constituents of the product.

Immunogenicity

Antibodies against components of fibrin sealant/haemostatic products may occur rarely.

However in a clinical trial with human fibrinogen/human thrombin sponge (patch) in hepatic surgery, in which patients were investigated for the development of antibodies, 26% of the 96 patients tested and treated with human fibrinogen/human thrombin sponge (patch) developed antibodies to equine collagen. The equine collagen antibodies that developed in some patients after human fibrinogen/human thrombin sponge (patch) use were not reactive with human collagen. One patient developed antibodies to human fibrinogen.

There were no adverse events attributable to the development of human fibrinogen or equine collagen antibodies.

There is very limited clinical data available regarding re-exposure of the human fibrinogen/human thrombin sponge (patch). Two subjects have been re-exposed in a clinical trial and have not reported any immune-mediated adverse events; however, their antibody status to collagen or fibrinogen is unknown.

Postmarketing

Following is a list of ADRs which have been observed in postmarketing and are not included above:

Immune system disorders: Anaphylactic shock, hypersensitivity
Vascular disorders: Thrombosis

Gastrointestinal disorders: Intestinal obstruction (in abdominal surgeries), ileus (in abdominal surgeries)

General disorders and administration site conditions: Adhesions

4.9 Overdose

No case of overdose has been reported.

5. PHARMACOLOGICAL PROPERTIES

5.1. Pharmacodynamic properties

Pharmaco-therapeutic group: Local haemostatics, ATC code: B02BC30

Tachosil contains fibrinogen and thrombin as a dried coating on the surface of a collagen sponge. In contact with physiological fluids, e.g. blood, lymph or physiological saline solution the components of the coating dissolve and partly diffuse into the wound surface. This is followed by the fibrinogen-thrombin reaction which initiates the last phase of physiological blood coagulation. Fibrinogen is converted into fibrin monomers which spontaneously polymerise to a fibrin clot, which holds the collagen sponge tightly to the wound surface. The fibrin is then cross linked by endogenous factor XIII, creating a firm, mechanically stable network with good adhesive properties and therefore provides sealing as well.

Clinical studies demonstrating haemostasis were conducted in a total of 240 patients (119 Tachosil, 121 argon beamer) undergoing partial liver resection surgery and 185 patients (92 Tachosil, 93 standard surgical treatment) undergoing surgical resection of superficial renal tumour. A further controlled study in 119 patients (62 Tachosil, 57 haemostatic fleece) demonstrated sealing, haemostasis and suture support in patients undergoing cardiovascular surgery. Tissue sealing in lung surgery was investigated in two controlled trials in patients undergoing lung surgery. The first controlled clinical trial investigating tissue sealing in lung surgery failed to document superiority over standard treatment measured by air leakage due to the inclusion of a large group of patients (53%) without air leakage. However, the second study investigating tissue sealing in 299 patients (149 Tachosil, 150 standard surgical treatment) with demonstrated intraoperative air leakage showed the superiority of Tachosil compared to standard treatment.

5.2. Pharmacokinetic properties

Tachosil is intended for epilesional use only. Intravascular administration is contraindicated.

As a consequence, intravascular pharmacokinetic studies were not performed in man.

Fibrin Sealants/haemostatics are metabolized in the same way as endogenous fibrin by fibrinolysis and phagocytosis.

In animal studies, Tachosil biodegrades after administration to a wound surface with few remnants left after 13 weeks. Complete degradation of Tachosil was seen in some animals 12 months after its administration to a liver wound, whereas small remnants were still observed in others. The degradation was associated with infiltration of granulocytes and formation of resorptive granulation tissue encapsulating the degraded remnants of Tachosil. No evidence of local intolerance has been observed in animal studies.

From the experience in humans there have been isolated cases where remnants were observed as coincidental findings with no signs of functional impairment

5.3 Non clinical Safety Data

Single dose and repeat dose toxicity studies in different species of animals have shown no signs of toxic effect.

6 PHARMACEUTICAL PARTICULARS

6.1 List of excipients

Equine collagen
Human albumin
Riboflavine (E 101)
Sodium chloride
Sodium citrate (E331)
L-arginine-hydrochloride.

6.2 Incompatibilities

Not applicable.

6.3 Shelf life

3 years.

Once the foil sachet is opened, Tachosil must be used immediately.

6.4 Special precautions for storage

Do not store above 25 °C.

6.5 Nature and contents of container

Each sponge is packed in a PET-GAG blister sealed with a coated PE foil. The blister is packed in an aluminium-bonded foil sachet with a desiccant bag included and packed in a folding carton.

Pack sizes:

Package with 1 sponge of 9.5 cm x 4.8 cm

Package with 2 sponges of 4.8 cm x 4.8 cm

Package with 1 sponge of 3.0 cm x 2.5 cm

Package with 5 sponges of 3.0 cm x 2.5 cm

Not all pack sizes may be marketed.

6.6 Special precautions for disposal and other handling

Tachosil comes ready to use in sterile packages and must be handled accordingly. Use only undamaged packages. Once the package is opened post-sterilisation is not possible. The outer aluminium foil sachet may be opened in a non-sterile operating area. The inner sterile blister must be opened in a sterile operating room area. Tachosil should be used immediately after opening the inner sterile cover.

Tachosil is used under sterile conditions. Prior to application the wound area should be cleansed, e.g. from blood, disinfectants and other fluids. After removal of Tachosil from the sterile package the sponge should be pre-moistened in saline solution and then applied immediately. It is important to note that failure to adequately clean adjacent tissues may cause adhesions. The yellow, active side of the sponge is applied to the bleeding/leaking surface and held against it with a gentle pressure for 3-5 minutes. This procedure enables an easy adhesion of Tachosil to the wound surface. Pressure is applied with moistened gloves or a moist pad. Due to the strong affinity of collagen to blood, Tachosil may also stick to surgical instruments, gloves or adjacent tissues covered with blood. This can be avoided by pre-moistening surgical instruments, gloves and adjacent tissues with physiological saline solution. After pressing Tachosil to the wound, the glove or the pad must be removed carefully. To avoid the sponge from being pulled loose it may be held in place at one end, e.g. with a pair of forceps.

Alternatively, e.g. in case of stronger bleeding, Tachosil may be applied without pre-moistening, while also pressing gently to the wound for 3-5 minutes.

The Tachosil sponge should be applied so that it extends 1-2 cm beyond the margins of the wound. If more than one sponge is used the sponges should overlap. The sponge can be cut to the correct size and shaped if too large.

Any unused medicinal product or waste material should be disposed of in accordance with local requirements.

7 NAME AND ADDRESS OF MANUFACTURER AND PRODUCT REGISTRATION HOLDER

Manufacturer
Corza Medical Distribution GmbH, Austria Branch
St. Peter Strasse 25
A-4020 Linz
Austria

Product Registration Holder:
PE PHARMA SDN. BHD.
15- 13A, Wisma UOA 2, Jalan Pinang 50450 Kuala Lumpur,
Wilayah Persekutuan Kuala Lumpur, Malaysia

8 DATE OF REVISION OF THE TEXT

13 January 2025 (CCDS v.1.0)